Pembrolizumab 400mg Monotherapy

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
First-line treatment of metastatic non-small cell lung carcinoma (NSCLC) in adults whose tumours express PD-L1 with a ≥50% tumour proportion score (TPS) with no EGFR mutations or ALK translocations.	C34	00558a	ODMS 01/04/2018
As monotherapy for the treatment of adults with unresectable or advanced melanoma.	C43	00558b	ODMS June 2016
For the treatment of ipilimumab-refractory patients with unresectable or advanced metastatic melanoma.	C43	00558c	ODMS June 2016
As monotherapy for the treatment of locally advanced or metastatic urothelial carcinoma in adults who have received prior platinum-containing chemotherapy.	C65, C66, C67, C68	00558e	ODMS 01/02/2021
As monotherapy is indicated for the treatment of locally advanced or metastatic urothelial carcinoma in adults who are not eligible for cisplatin-containing chemotherapy whose tumours express PD-L1 with a combined positive score (CPS) ≥10.	C65, C66, C67, C68	00558f	ODMS 01/02/2021
As monotherapy is indicated for the adjuvant treatment of adults with Stage III melanoma and lymph node involvement who have undergone complete resection	C43	00558g	ODMS 01/05/2021
As monotherapy for the first-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma (HNSCC) in adults whose tumours express PD-L1 with a CPS ≥ 1.	C00-C14, C30-C32, C76	00558h	ODMS 20/12/2021
As monotherapy for the treatment of adult patients with relapsed or refractory (R/R) classical Hodgkin lymphoma (cHL) who have failed autologous stem cell transplant (ASCT) or following at least two prior therapies when ASCT is not a treatment option.	C81	00558i	ODMS 01/02/2022
For the maintenance treatment of adult patients with persistent, recurrent, or metastatic cervical cancer whose tumours express PD-L1 with a CPS ≥ 1 who have completed the induction chemotherapy component of the treatment	C53	00558j	ODMS 01/08/2026
First-line treatment of metastatic microsatellite instability-high (MSI-H) or mismatch repair deficient (dMMR) colorectal cancer (CRC) in adults.	C18, C19, C20	00558k	ODMS 01/04/2023
As monotherapy for the adjuvant treatment of adults with Stage IIB or IIC melanoma and who have undergone complete resection.	C43	00558l	ODMS 01/12/2024
For the adjuvant treatment of adult patients with renal cell carcinoma (RCC) at increased risk of recurrence following nephrectomy, or following nephrectomy and resection of metastatic lesions.	C64	00558m	ODMS 01/12/2024
As monotherapy for the adjuvant treatment of adults with non-small cell lung carcinoma who are at a high risk of recurrence following complete resection and platinum-based chemotherapy	C34	00558n	N/A

*This is for post 2012 indications only

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TREATMENT:

The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patient's individual clinical circumstances.

Pembrolizumab is given on day 1 of each cycle.

Cycle frequency and duration:

Each cycle is 42 days.

Atypical responses (i.e., an initial transient increase in tumour size or small new lesions within the first few months followed by tumour shrinkage) have been observed. It is recommended to continue treatment for clinically stable patients with initial evidence of disease progression until disease progression is confirmed.

For adjuvant treatment of melanoma, renal cell carcinoma (RCC) and NSCLC, the maximum treatment duration with pembrolizumab is 12 months.

For all other indications, pembrolizumab is continued until disease progression or unacceptable toxicity develops.

At 24 months, discontinuation may be considered in patients with a sustained response and no evidence of progression. However, discontinuation is not mandatory, and treatment may be continued where clinically appropriate.
Please refer to the **Exclusions** section for the link to *Information regarding prior therapy with an anti-PD-1 or anti-PD-L1 antibody*, which includes information on treatment breaks.

Facilities to treat anaphylaxis MUST be present when pembrolizumab is administered.

Day	Drug	Dose	Route	Diluent & Rate	Cycle
1	Pembrolizumab	400mg	IV infusion	100mL 0.9% NaCl over 30 minutes	Every 42 days (6 weeks)
Pembrolizumab is diluted to a final concentration ranging from 1-10mg/ml.					
Administer using a low-protein binding 0.2 to 5 micrometre in-line or add-on filter.					

Note: Administration volumes and fluids have been standardised to facilitate electronic prescribing system builds.

ELIGIBILITY:

- Indications as above
- Adequate haematological, hepatic and renal function
- **First line Non-Small Cell Lung Cancer**
  - Histologically or cytologically confirmed stage IV NSCLC with no sensitizing EGFR mutations or ALK translocations
  - ECOG status 0-1
  - Confirmation of PD-L1 tumour proportion score of 50% or greater by a validated test
  - No previous systemic therapy for metastatic disease

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- **Melanoma**
  - Advanced:
    - No more than one previous systemic treatment for advanced disease
    - ECOG status 0-1
  - Adjuvant Stage III:
    - Melanoma with involvement of lymph nodes or metastatic disease who have undergone complete resection
    - ECOG status 0-1
  - Adjuvant Stage IIB/IIC:
    - Confirmed new diagnosis of Stage IIB or IIC cutaneous melanoma per American Joint Committee (AJCC) on Cancer 8th edition guidelines
    - Adjuvant pembrolizumab should start within 12 weeks of surgery
    - ECOG status 0-1
- **Classical Hodgkin Lymphoma**
  - Consider the benefit of treatment with pembrolizumab versus the risk of possible GVHD in patients with a history of allogeneic HSCT
  - ECOG status 0-1

- **Urothelial carcinoma second- line:**
  - Histologically or cytologically confirmed urothelial carcinoma of the renal pelvis, ureter, bladder or urethra that shows predominantly transitional-cell features on histologic testing
  - ECOG 0-2
  - Have had progression or recurrence of urothelial cancer following receipt of a first-line platinum-containing regimen (CISplatin or CARBOplatin)
- **Urothelial carcinoma first-line**
  - Histologically- or cytologically-confirmed diagnosis of advanced/unresectable (inoperable) or metastatic urothelial cancer of the renal pelvis, ureter, bladder, or urethra (transitional cell and mixed transitional/non-transitional cell histologies)
  - Ineligible for CISplatin therapy
  - ECOG 0-2
  - PD-L1 with a combined positive score (CPS) >10 as demonstrated by a validated assay method
- **Head and neck squamous cell carcinoma (HNSCC)**
  - Histologically or cytologically-confirmed recurrent or metastatic head and neck squamous cell carcinoma considered incurable by local therapies
  - Primary HNSCC tumour excluding cancers of the nasopharynx
  - ECOG 0-2
  - PD-L1 with a combined positive score (CPS) ≥1 as demonstrated by a validated assay method
- **Cervical:**
  - ECOG 0-2
  - PD-L1 with a combined positive score (CPS) ≥1 as demonstrated by a validated test method

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- **Metastatic colorectal cancer:**
  - ECOG 0-2
  - Histologically confirmed dMMR/MSI-high CRC as demonstrated by a validated test method
- **Renal cell carcinoma:**
  - Histologically confirmed diagnosis of clear cell RCC
  - ECOG 0-2
  - Treatment should start within 12 weeks of surgery
- **Adjuvant NSCLC:**
  - Pathologically confirmed NSCLC of stage IB-IIIA per the AJCC staging system (7th edition) after complete surgical resection.
  - ECOG 0-1
  - Prior adjuvant chemotherapy

CAUTION:

- History of serious autoimmune disease
- Immunosuppressive doses of systemic corticosteroids or other immunosuppressive medication(s) (defined as >10mg prednisoLONE/daily (or steroid equivalent, excluding inhaled or topical steroids)
- Any active clinically significant infection requiring therapy
- Untreated brain metastases
- History of interstitial lung disease

EXCLUSIONS:

- Hypersensitivity to pembrolizumab or any of the excipients
- Information regarding prior therapy with an anti PD-1 or anti PD-L1 antibody is Available on the NCCP website
- HNSCC: Progressive disease within six months of completion of curatively intended systemic treatment for locoregionally advanced HNSCC

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant Medical Oncologist or Consultant Haematologist experienced in the treatment of haematological malignancies

TESTS:

Baseline tests:

- FBC, renal and liver profile
- Blood glucose
- Thyroid function tests.
- 1L NSCLC, 1L urothelial cancer, HNSCC, cervical cancer: PD-L1 expression using a validated test method

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Regular tests:

- FBC, renal and liver profile prior to each cycle
- Blood glucose prior to each cycle
- Thyroid Function Tests every 3 to 6 weeks
- Cortisol as clinically indicated

Disease monitoring:
Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

DOSE MODIFICATIONS:

- Any dose modification should be discussed with a Consultant.
- Management of immune-related adverse reactions may require withholding of a dose or permanent discontinuation of pembrolizumab therapy and institution of systemic high-dose corticosteroid.
- Dose reduction is not recommended.
- Guidelines for withholding of doses or permanent discontinuation are described below in Table 1.

Table 1: Recommended treatment modifications for pembrolizumab

Immune-related adverse reactions	Severity (NCI-CTCAE v.4 grading)	Treatment modification
Pneumonitis	Grade 2	Withhold*
	Grade 3 or 4, or recurrent Grade 2	Permanently discontinue
Colitis	Grade 2 or 3	Withhold*
	Grade 4 or recurrent Grade 3	Permanently discontinue
Nephritis	Grade 2 with creatinine >1.5 to ≤3 times upper limit of normal (ULN)	Withhold*
	Grade ≥3 with creatinine >3 times ULN	Permanently discontinue
Endocrinopathies	Grade 2 adrenal insufficiency and hypophysitis	Withhold treatment until controlled by hormone replacement
	Grades 3 or 4 adrenal insufficiency or symptomatic hypophysitis Type 1 diabetes associated with Grade ≥3 hyperglycaemia (glucose >250 mg/dL or >13.9 mmol/L) or associated with ketoacidosis Hyperthyroidism Grade ≥3	Withhold* For patients with Grade 3 or Grade 4 endocrinopathy that improved to Grade 2 or lower and is controlled with hormone replacement, if indicated, continuation of pembrolizumab may be considered after corticosteroid taper, if needed. Otherwise treatment should be discontinued

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	Hypothyroidism	Hypothyroidism may be managed with replacement therapy without treatment interruption
Hepatitis	Grade 2 with aspartate aminotransferase (AST) or alanine aminotransferase (ALT) >3 to 5 times ULN or total bilirubin >1.5 to 3 times ULN	Withhold*
	Grade ≥3 with AST or ALT >5 times ULN or total bilirubin >3 times ULN	Permanently discontinue
	In case of liver metastasis with baseline Grade 2 elevation of AST or ALT, hepatitis with AST or ALT increases ≥50% and lasts ≥1 week	
Skin reactions	Grade 3 or suspected Stevens-Johnson syndrome (SJS) or toxic epidermal necrolysis (TEN)	Withhold*
	Grade 4 or confirmed SJS or TEN	Permanently discontinue
Other immune-related adverse reactions**	Based on severity and type of reaction (grade 2 or Grade 3)	Withhold*
	Grade 3 or4 myocarditis Grade 3 or 4 encephalitis Grade 3 or 4 Guillain-Barre syndrome Grade 4 or recurrent Grade 3	Permanently discontinue
Infusion-related reactions	Grade 3 or 4	Permanently discontinue

* Until adverse reactions recover to Grade 0-1. If treatment related toxicity does not resolve to Grade 0-1 within 12 weeks after last dose of pembrolizumab or if corticosteroid dosing cannot be reduced to ≤ 10mg prednisoLONE or equivalent per day within 12 weeks, pembrolizumab should be permanently discontinued

**Pembrolizumab should be permanently discontinued for Grade 4 or recurrent Grade 3 immune-related adverse reactions, unless otherwise specified in Table 1.

Renal and Hepatic Impairment:

Table 2: Dose modification of pembrolizumab in renal and hepatic impairment

Renal Impairment		**Hepatic Impairment**	
No dose adjustment is needed		**Mild**	No dose adjustment is needed
Haemodialysis	No need for dose adjustment is expected	**Moderate/Severe**	No need for dose adjustment is expected
Renal and hepatic recommendations: Giraud et al, 2023			

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SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting- Available on the NCCP website

Pembrolizumab Minimal (**Refer to local policy).**

For information:
Within NCIS regimens, antiemetics have been standardised by Medical Oncologists and Haemato-oncologists and information is available in the following documents:

- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Medical Oncology) - Available on the NCCP website
- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Haemato-oncology) - Available on the NCCP website

PREMEDICATIONS: None usually required

OTHER SUPPORTIVE CARE:

- Women of childbearing potential should use effective contraception during treatment with pembrolizumab and for at least 4 months after the last dose of pembrolizumab.

ADVERSE EFFECTS

- Please refer to the relevant Summary of Product Characteristics(SmPC) for full details.
- Please note the NCCP National SACT regimens do not provide information regarding black triangle (▼) status. Please refer to the relevant Summary of Product Characteristics (SmPC) for this information.

DRUG INTERACTIONS:

- Current SmPC and drug interaction databases should be consulted for information.

COMPANY SUPPORT RESOURCES/Useful Links:

Please note that this is for information only and does not constitute endorsement by the NCCP

Patient Card
https://assets.hpra.ie/products/Human/10089/094590ae-1f3d-4b15-b76e-3b16bd642782.pdf

REFERENCES:

1. Bellmunt, J et al. Pembrolizumab as Second-Line Therapy for Advanced Urothelial Carcinoma. N Engl J Med 2017; 376:1015-26.
2. Balar, A et al. First-line pembrolizumab in cisplatin-ineligible patients with locally advanced and unresectable or metastatic urothelial cancer (KEYNOTE-052): a multicentre, single-arm, phase 2 study. Lancet Oncol 2017; 18: 1483–92

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3. Lala M, Li M, Sinha V, de Alwis D, Chartash E, Jain L. A six-weekly (Q6W) dosing schedule for pembrolizumab based on an exposure-response (E-R) evaluation using modeling and simulation. J Clin Oncol. 2018; 36 (suppl; abstr 306). doi: 10.1200/JCO.2018.36.15_suppl.3062.
4. Reck M, Delvys R-A et al. Pembrolizumab versus Chemotherapy for PD-L1-Positive Non-Small-Cell Lung Cancer. N Engl J Med 2016;375:1823-1833.
5. Robert C, et al. Pembrolizumab versus Ipilimumab in Advanced Melanoma. N Engl J Med 2015;372:2521-2532.
6. Ribas A, et al. Pembrolizumab versus investigator-choice chemotherapy for ipilimumab-refractory melanoma (KEYNOTE-002): a randomised, controlled, phase 2 trial. Lancet Oncology 2015; 16 ;(8)908-918.
7. Robert C, et al. Anti-programmed-death-receptor-1 treatment with pembrolizumab in ipilimumab-refractory advanced melanoma: a randomised dose-comparison cohort of a phase 1 trial**.** Lancet Oncol 2014; 384:1109-17.
8. Chen R et al. Phase II Study of the Efficacy and Safety of Pembrolizumab for Relapsed/Refractory Classic Hodgkin Lymphoma. J Clin Oncol 2017;35:2125-2133
9. Gutzmer, R. et al. Programmed cell death protein-1 (PD-1) inhibitor therapy in patients with advanced melanoma and preexisting autoimmunity or ipilimumab-triggered autoimmunity. European Journal of Cancer; 2017, 75, 24–32. https://doi.org/10.1016/j.ejca.2016.12.038
10. Eggermont, A. et al. 2018. Adjuvant Pembrolizumab versus Placebo in Resected Stage III Melanoma. N Engl J Med 378(19):1789-1801.
11. Burtness B, et al Pembrolizumab alone or with chemotherapy versus cetuximab with chemotherapy for recurrent or metastatic squamous cell carcinoma of the head and neck (KEYNOTE-048): a randomised, open-label, phase 3 study. Lancet; Available from: http://www.ncbi.nlm.nih.gov/pubmed/31679945
12. Marabelle A, et al. Efficacy of Pembrolizumab in Patients with Noncolorectal High Microsatellite Instability/Mismatch Repair-Deficient Cancer: Results from the Phase II KEYNOTE-158 Study. J Clin Oncol 2020;38(1):1-10.
13. Andre T, et al. Pembrolizumab in Microsatellite-Instability-High Advanced Colorectal Cancer. N Engl J Med 2020; 383:2207-2218.
14. Luke J et al. Pembrolizumab versus placebo as adjuvant therapy in completely resected stage IIB or IIC melanoma (KEYNOTE-716): a randomised, double blind, phase 3 trial. Lancet 2022 Apr 30;399(10336):1718-1729
15. Luke JJ, Ascierto PA, Khattak MA, et al. Pembrolizumab Versus Placebo as Adjuvant Therapy in Resected Stage IIB or IIC Melanoma: Final Analysis of Distant Metastasis-Free Survival in the Phase III KEYNOTE-716 Study. *J Clin Oncol*. 2024;42(14):1619-1624.
16. Choueiri TK et al. Adjuvant Pembrolizumab after Nephrectomy in Renal-Cell Carcinoma. N Engl J Med 2021;385:683-694
17. O' Brien M et al. Pembrolizumab versus placebo as adjuvant therapy for completely resected stage IB–IIIA non-small-cell lung cancer (PEARLS/KEYNOTE-091): an interim analysis of a randomised, triple-blind, phase 3 trial. Lancet Oncol 2022:23:1274-86. Available here: https://pubmed.ncbi.nlm.nih.gov/36108662/
18. Giraud E L, Lijster B D, et al. Dose recommendations for anticancer drugs in patients with renal or hepatic impairment: an update. Available at: https://pubmed.ncbi.nlm.nih.gov/37269847/
19. NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting. V6 2025.. Available at: https://www.hse.ie/eng/services/list/5/cancer/profinfo/chemoprotocols/nccp-classification-document-for-systemic-anti-cancer-therapy-sact-induced-nausea-and-vomiting.pdf

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20. Pembrolizumab (Keytruda®) Summary of Product Characteristics. Updated 22/04/2026 Accessed May 2026. Available at: https://www.ema.europa.eu/en/documents/product-information/keytruda-epar-product-information_en.pdf

Version	Date	Amendment	Approved By
1	10/04/2019		Dr Deirdre O'Mahony Prof Michaela Higgins
2	10/07/2019	Update of indication for 00558b	Prof Maccon Keane
3	21/08/2019	Addition of first line and second line indications for urothelial cancer	Prof Maccon Keane
4	23/09/2020	Updated management of adverse events in line with SmPC update. Addition of adjuvant melanoma indication.	Prof Maccon Keane
5	01/02/2021	Updated reimbursement status	Prof Maccon Keane
6	30/04/2021	Updated indication for 00558g Updated reimbursement status	Prof Maccon Keane
7	09/09/2021	Reviewed. Amended Table 1 (symbols re nephritis and endocrinopathies). Updated company support resources.	Prof Maccon Keane
8	22/12/2021	Updated indication for 00558h Updated reimbursement status Updated table 1 in line with SmPC.	Dr Cliona Grant
9	26/01/2022	Updated: deactivation of 00558d and inclusion of indication 00558i. Updated reimbursement status.	Prof Maccon Keane
10	01/04/2023	Addition of cervical and mCRC indications	Prof Maccon Keane
11	13/08/2024	New indication added (00558l). Exclusions section updated with regard to previous PD-1/PD-L1 treatment. Renal and hepatic dose modifications aligned to Giraud et al 2023 recommendations. Regimen updated as per NCCP standardisation.	Prof Fergal Kelleher
12	12/09/2024	Adjuvant RCC indication added. Cautions and exclusions sections amended.	Prof Maccon Keane
12a	29/11/2024	HSE Reimbursement status updated of indications 00558l and 00558m.	NCCP
12b	08/05/2025	Update to ICD-10 codes for head and neck, urothelial and colorectal cancer	NCCP
13	24/07/2026	New indication added- 0558n Indication 0558j updated. Treatment section, eligibility, cautions, exclusions, baseline tests updated.	Prof Maccon Keane

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